



Physiotherapy & Rehabilitation Governance Platform

Medical Report: Post-ACL Reconstruction Rehabilitation

Patient Information

Name: ahmet kalumian

Birth Date: 2025-10-28

Case ID: #24

Clinical Details

Doctor: Dr. PSP

Date: 2026-03-28

Description: A 28-year-old male athlete, 4 weeks post-op for ACL reconstruction using a hamstring graft. Currently experiencing mild swelling and limited range of motion (ROM) in the right knee.

Physical Measurements

Height: 182 cm

Weight: 78 kg

Blood Group: OPositive

Health History

Smoker: No

Chronic Diseases: No

Activity Level: Active

Additional Patient Info

InjuryHistory: Complete ACL tear occurred during a football match on January 15, 2026, due to a sudden pivoting motion. Surgery was performed on March 1, 2026.

Medications: Ibuprofen 400mg as needed for pain, Vitamin D3 supplements, and Omega-3 fish oil.

FunctionalAbility: Partial weight-bearing with one crutch. Knee flexion is currently at 95 degrees. Struggles with stairs and long-distance walking.

PersonalGoals: Full return to competitive football within 9 months, restoration of full quad strength, and achieving 135-degree knee flexion.

Smart Clinical Analysis

Clinical Explanation:

The patient is a 28-year-old male athlete who is 4 weeks post-operative following ACL reconstruction using a hamstring graft. He presents with mild swelling and limited range of motion in the right knee, which is common during the early stages of rehabilitation. The patient has a functional ability of partial weight-bearing with the assistance of one crutch and is currently experiencing knee flexion of 95 degrees. His goals include a full return to competitive football within 9 months, restoration of full quadriceps strength, and achieving 135 degrees of knee flexion.

Immediate Physiotherapy Recommendations:

The focus of physiotherapy should include gentle range of motion exercises to improve knee flexion, along with isometric quadriceps strengthening exercises. Gradual progression to weight-bearing activities should be encouraged as tolerated, while monitoring for any signs of increased swelling or pain. Incorporating modalities such as ice and elevation may assist in managing swelling.

Precautions:

The patient should avoid high-impact activities, sudden pivoting motions, and any exercises that cause significant pain or discomfort. Close monitoring of knee swelling and pain levels is essential, and any increase in symptoms should prompt a reassessment of the rehabilitation plan.

Estimated Recovery Timeline:

Given the current progress and the patient's goals, a full recovery to competitive football may be anticipated within 9 months, contingent upon adherence to rehabilitation protocols and absence of complications.

Risk Assessment:

Potential risks include re-injury of the ACL, development of stiffness in the knee joint, and delayed healing due to inadequate rehabilitation. It is crucial to ensure that the patient follows the prescribed rehabilitation plan to mitigate these risks.

Red Flags:

Emergency warning signs include severe swelling, increased pain not relieved by medication, inability to bear weight on the affected leg, signs of infection (such as fever or increased redness), and any sudden changes in sensation or function of the limb.

Test Group: Range of Motion (ROM) Assessment

Test: Knee Flexion (Right)

Clinical AI Analysis

Normal Reference:

Normal knee flexion for a healthy adult is typically between 130 to 150 degrees.

Clinical Interpretation:

The patient currently demonstrates knee flexion of 95 degrees, which is below the normal range. However, there is a noted improvement in flexion from 95 degrees to 115 degrees over the assessment period, indicating a positive trend in recovery.

Progress Evaluation:

The trend shows a gradual increase in knee flexion, with measurements progressing from 95 degrees to 115 degrees. This suggests effective rehabilitation efforts and a positive response to physiotherapy interventions.

Physiotherapy Focus:

Continue to emphasize range of motion exercises, focusing on achieving greater knee flexion while ensuring that the patient does not experience excessive pain or swelling during activities.

HIGHEST

115.0

LOWEST

95.0

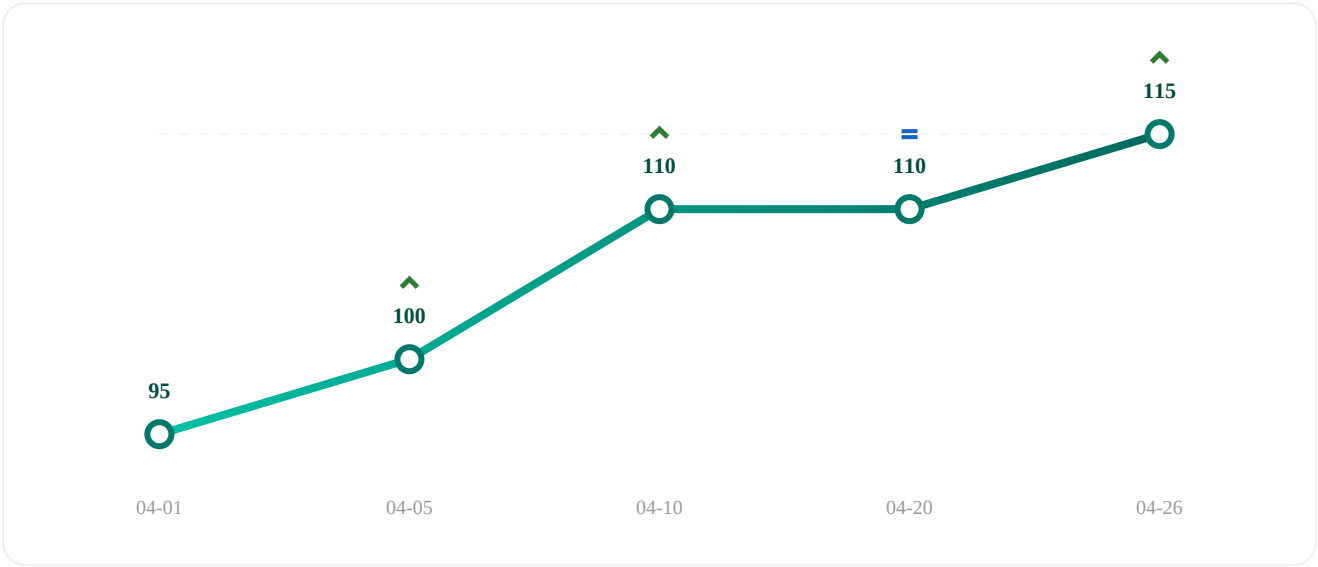
AVERAGE

106.0

OVERALL PROGRESS

Increased by 21.1% ↑

Since first test (2026-04-01)



Date	Result
2026-04-01	95
2026-04-05	100
2026-04-10	110
2026-04-20	110
2026-04-26	115